

Finding Counties Missed by Infrastructure-Only Medicaid Access Screening

Postpartum Medicaid Access Barrier Index

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Research question: Where may postpartum Medicaid populations face greater administrative access barriers to maintaining coverage and navigating support after delivery?

This brief summarizes a county-level screening index designed to identify where administrative, clinical, and access-support barriers relevant to postpartum Medicaid access may overlap.

Main Finding

An **infrastructure-only screen** looks for counties with both major local infrastructure gaps: no Medicaid office and no hospital-based obstetric care. This screen identifies many high-concern counties, but it is incomplete.

Measure	Result
Counties classified as high or highest concern	665
High/highest counties captured by the infrastructure-only screen	386
High/highest counties missed by the infrastructure-only screen	279

The missed counties were not low-burden outliers. They had an average score of 6.46 and an average contextual support-burden score of 4.56. Most were nonmetro, and large shares had high poverty, high no-internet rates, high disability rates, or high no-vehicle household rates.

What the Index Adds

The index does not only ask whether a county has both major infrastructure gaps. It distinguishes three domains:

- **Administrative access:** whether the county has a Medicaid office.
- **Clinical maternity infrastructure:** whether the county has hospital-based obstetric care.
- **Access-support burden:** whether contextual barriers may make administrative or clinical gaps harder to navigate.

This matters because a county with only one infrastructure gap may still face substantial access burden if that gap overlaps with poverty, transportation barriers, digital access barriers, limited English access needs, disability-related support context, reproductive-age population context, or rurality.

Score Design

The Postpartum Medicaid Access Barrier Index is a transparent county-level screening score. The theoretical score range is 0-11, and observed county scores ranged from 0 to 10.

- No Medicaid office in county = 2 points.
- No hospital-based obstetric care = 2 points.
- Top quartile poverty rate = 1 point.

- Top quartile no-vehicle household rate = 1 point.
- Top quartile no-internet subscription rate = 1 point.
- Top quartile limited English rate = 1 point.
- Top quartile disability rate = 1 point.
- Top quartile female population ages 15-44 rate = 1 point.
- Nonmetro county status = 1 point.

Practical Review Sequence

1. Start with highest-concern counties.

Review the 56 highest-concern counties first, including the 11 counties with the maximum observed score of 10.

2. Review counties with both infrastructure gaps.

Counties with both no Medicaid office and no hospital-based obstetric care are the clearest infrastructure-gap counties.

3. Review one-gap plus high-context counties.

These counties may not appear in an infrastructure-only screen but still have multiple access-support barriers.

4. Review contextual-burden watchlist counties.

A smaller group reaches high concern through contextual burden without either direct infrastructure gap.

Applied Screening Rule

Among the 279 high/highest concern counties missed by the infrastructure-only screen, a rule requiring **one infrastructure gap plus at least four contextual flags** recovered 265 counties, or 95.0%, with 100.0% precision.

This finding supports the two-layer structure of the index. Direct infrastructure gaps matter, but contextual burden helps identify which one-gap counties may deserve closer review.

Geographic Pattern

High scores were concentrated in the South, parts of the Southwest, and Alaska. Mississippi had the highest average county score and the highest share of high/highest concern counties. Texas had the largest number of high/highest concern counties.

These patterns identify a geography for follow-up analysis using Medicaid enrollment, renewal, churn, coverage continuity, and postpartum care data.

Bottom Line

The most obvious infrastructure gaps are only part of the access-burden picture. The index provides a transparent way to identify counties that may be overlooked by simpler Medicaid access screens and to show where additional barriers make those counties worth closer review.

Repository: <https://github.com/caroline-howard/medicaid-access-barriers-powerbi>